



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : فاطمه رزق اسماعيل شافعى

المحافظة : الجيزة الحي / المدينة : الجيزة

العنوان : قرية ام دينار

تاريخ تسجيل الإستمارة : 10-10-2026

رقم البطاقة : 27409082101184 رقم التليفون : 01555223180

العمر (تاريخ الميلاد) : 08-09-1974 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د/..... د/..... د/..... د/.....

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: فاطمه رزق اسماعيل شافعي National ID: 27409082101184	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Bilateral mammogram and breast ultrasound
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



أرملة

مستفيدة

التي

البطاقة سارية حتى ٢٠٣١/١٠/٠٤



٢٠٢٤/١٠ ٣٧٤٠٩٠٨٢١٠١١٨٤

زينة منزل



جمهورية الكويت
Ministry of Social Affairs

فاطمة

رزق اسماعيل شفيق

قرية ام ديلر

منطقة القحط - الجيزة

٢٧٤٠٩٠٨٢١٠١١٨٤



KU0795564

Dms

MDT FORM

Patient Name	فاطمة رزق اسماعيل	Age	50
Sex	F	Special habits	
Family History		Comorbidity	
Surgical History	Hysterectomy	Telephone	
Menopausal Status	Postmenopausal	Marital Status	
ID	27409082101184	Medical ID	1500/25
Date	21/9/2025	Reason of MDT	

Medical Report

Pathology: Bx-> IDC G2

ER: 8/8 PR:8/8 HER 2:0 ki67 : 10%

pT3 pN1 M0

S.M: LT united malignant mass with ductal extension reaching the nipple

Metsworkup:

CT CAP :FREE

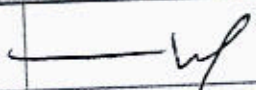
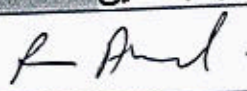
Recommendations:

UPFRONT LT simple mastectomy + SLNB

IDC 90% WITH FOCAL ILC 10% DCIS 5% and (1/10) with focal nodal extension

MDT DECISION

For adjuvant AC x4 -> Taxol weekly x12 weeks -> PORT
-> AI +zoladex x 7 years
to start zoladex with chemo

Name	د/ حنان عزت	د/ رانيا شفيق
Signature		

Name: فاطمه رزق اسماعيل شافعي		
MR. : 2784		
Detailed Medical Report		
		السيد
تحية طيبة وبعد,,		
إيماء للطلب المقدم من	بشأن تقرير طبي للمريض	
بقسم:	رقم طبي:	
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
تاريخ وساعة الدخول :	تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه		
MDT discussion for 4 cycles of AC and 12 weeks of TAXOL; first week of TAXOL (C1 W1) dose 70 mg/m² based on weight 71 kg and height 150 cm, BSA 1.72 m²; second week (W2) dose 3 mg (pharmacy note). Patient presented for week 1 TAXOL 135 mg; labs were acceptable. Week 2 and week 3 TAXOL doses were administered and pharmacy was notified. Pharmacy visit for week 2 TAXOL 135 mg; patient missed the dose last week. Pharmacy visit for week 3 TAXOL 135 mg; weight 71 kg, height 150 cm, BSA 1.72 m²; labs on 5/4 were accepted. Medical oncology note: unknown menopausal status (hysterectomy performed); left breast cancer, stage 1/9/2025, IDC2, PT3, N1/10 mo, ER8/8, PR8/8, HER2-negative, Ki-67 10 %; 4 cycles of AC for ECG for week 5 TAXOL 135 mg (SA 1.7). Pharmacy visit for week 7 TAXOL 135 mg (dose reduction); labs on 4/6 were accepted. Medical oncology note: for week 9 TAXOL 120 mg (10 % dose reduction) due to grade 2 diarrhea and pain, adjuvant therapy. The patient has completed 9 weeks of TAXOL therapy to date.		
تحريراً في ,		
التاريخ :	20-09-2026	الوقت :
الطبيب المعالج :	dina makram	
يعتمد ,		

Patient Name : الأستاذة / فاطمة رزق اسماعيل شافعي	Request No. : 13007747
Patient No. : 27409082101184	Request Date : 17/04/2025
Path No. : 1382/25	Verification Date : 03/05/2025
Sex : Female	Financial Category : اورام دار السلام - مبادره
Age : 50 Y	Department : Out Patient
Referreing Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical data:

Left breast mass.

Nature of specimen:

Tru cut biopsy, collected at 16/4/2025.

Gross examination

- Four grey white tissue cores measured up to 1.2 cm, totally submitted.
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation +nuclear pleomorphism+mitosis): (3+2+1) Grade II .
- * DCIS: Focal.
- * Angioinvasion: Could not be detected within the limits of biopsy recieved.
- * Perineural invasion: Not detected.
- * IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- Her2: No membrane reaction in tumor cells (score 0).
- Ki67: 10% nuclear reaction of tumor cells.

Diagnosis:

Left breast mass tru cut biopsy:

Verified by :

Dr.Menan Hossam

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Page 1 of 3

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name :	الأستاذة / فاطمه رزق اسماعيل شافعي	Request No. :	13006082
Patient No. :	27409082101184	Request Date :	01/09/2025
Path No. :	2702/25	Verification Date :	11/09/2025
Sex :	Female	Financial Category :	جوستاف روسي - نفقة الدولة
Age :	50 Y	Department :	Out Patient
Referring Doc :		Registered by :	.marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer, Up-Front.

Nature Specimen: Left simple mastectomy & sentinel lymph nodes biopsy.

Gross Examination:

- * Left simple mastectomy specimen measured 24x20x5 cm, with covering skin ellipse measured 18x15 cm. Nipple & Areola looked unremarkable grossly.
- * Serialling showed one mass measured 6x4x3 cm, located 1.5 cm from the deep margin.
- * Sentinel lymph nodes: Fibrofatty tissue piece measured 5x5 cm, entangling 10 lymph nodes, the largest was 1.5x1x0.5 cm (Only six were submitted for frozen & the rest were for paraffin).

12 Blocks:

- 1:9- Mass., 10- Nipple & areola., 11- Deep margin., 12- Frozen SLNs., 13&14- Paraffin SLMs.
- Gross was done by; Dr. Heba Hassan.

* Frozen Results:

- Positive metastatic deposits in one out of six sentinel lymph nodes (Ten lymph nodes were detected, but only six were submitted for frozen).

Microscopic Examination:

- * Histological Type: Invasive duct carcinoma (90%) with focal invasive lobular component (10%).
- * Grade 2 (differentiation 2, nuclear pleomorphism 2, mitosis 2).
- * 5% Intermeiate grade ductal carcinoma in situ (Solid pattern).
- * Angioinvasion: Present.
- * Perineural invasion: Present.
- * Nipple & areola: Free.
- * Deep margin: Free.

Verified by :

Dr.Duaa Mohamed

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Page 1 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / فاطمه رزق اسماعيل شافعي	Request No. : 13006082
Patient No. : 27409082101184	Request Date : 01/09/2025
Path No. : 2702/25	Verification Date : 11/09/2025
Sex : Female	Financial Category : جوستاف روسي - نفقة الدولة
Age : 50 Y	Department : Out Patient
Referring Doc :	Registered by : .marwa mohamed youss

Pathology Report

* Sentinel lymph nodes: Positive metastatic deposits in one out of ten lymph nodes (1/10) with focal extra-nodal extension.

Diagnosis:

Left breast cancer (Up-front), Left simple mastectomy & lymph nodes biopsy:

Invasive duct carcinoma (90%) with focal invasive lobular component (10%), Grade 2.

- 5% Intermediate grade ductal carcinoma in situ (Solid pattern).

- Deep margin: Free.

- Nipple & areola: Free.

- Sentinel lymph nodes: Positive metastatic deposits in one out of ten lymph nodes (1/10) with focal extra-nodal extension.

- Stage: pT3, pN1a.

Signature

Dr. Walaa Ibrahim Ghoneim.

Dr. Heba El-Shafeey.

Verified by : 

Dr.Duaa Mohamed

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name: Fatma Rezk Ismail Shafie	Date : 12/08/2025
Exam : MAMMOGRAPHY BOTH	Age : 51 Year(s)
Medical ID: 1500/2025	Pat. ID: 100433

- 11 o'clock, posterior third non-specific intra-mammary lymph node measuring 7 x 2 mm with 1.5 mm cortical thickness
- Normal skin/ and subcutaneous tissue.
- Non-specific axillary LNs.. No pathological infra or supra-clavicular lymph nodes.

Conclusion:-

Right breast (BIRADS 2): UIQ non-specific intra-mammary lymph node.

Left breast (BIRADS 6): 1 o'clock unifocal malignant mass lesion with suggested anterior ductal extension reaching the nipple and UIQ non-specific intra-mammary lymph node .

Best Regards,

DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

<u>Category & recommendations</u>	<u>Suspected Cancer Risk</u>
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routine screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	

Signature Dr. Amal Hegazy
PM 6:00 2025/08/12

Patient Name: Fatma Rezk Ismail Shafie	Date : 09/07/2025
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 51 Year(s)
Medical ID: 1500/2025 690 800	Pat. ID: 100433

***POST -CONTRAST CT SCAN OF THE CHEST ,
ABDOMEN & PELVIS***

Findings:

Known case of Breast cancer

- Early interstitial pulmonary changes are seen involving both lungs manifested by subpleural lines with interstitial coarsening & fine reticular infiltrations associated with mild septal thickening at both upper lobes
- No pleural effusion , calcification and masses.
- The cardiac size is not enlarged with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Unremarkable non-enhanced appearance of the thoracic aorta and the great vessels.
- **Mild enlarged liver no focal lesion.**
- **Average size spleen , no focal lesions.**
- *Average size gall bladder , no gall stones.
- *No dilated intra or extra-hepatic bile ducts .
- *Patent main portal vein trunk and both branches.
- *Urinary bladder shows no masses or stones.
- *Normal C.T. appearance of the pancreas , adrenal glands, both kidneys (apart right midcortical renal cyst), bowel loops & other pelvic organs.
- *No retroperitoneal lymphadenopathy .
- *No ascites .
- No worrisome lytic or sclerotic lesions,

Opinion:

- ***CT findings are impressive of early interstitial lung changes with apical fibrosis as forementioned for clinico-laboratory correlation.***
- ***Hepatomegaly , no focal lesion.***

Kindly accept my best wishes



الريادة للأشعة والتحليل

أ.د. عمر حسين عمر
أ.د. حسام فهمي عبد الحميد
أ.د. أحمد مصطفى
أ.د. طارق شوقي القاضي
أ.د. سامح محمد عبد الوهاب
أ.د. حسن عبد المنعم
أ.د. معتز سامي البيلاوي
أ.د. عمرو يوسف أيوب

Name:	Fatma Rezk Ismail Elshafey	Age :	50
Referred by:	Prof. Dr/ Ramy Gomaa Abdalwanes	Date :	14/04/2025
Examination:	MAMMOGRAPHY		

BILATERAL SONO-MAMMOGRAPHY:

The right breast is showing normal distribution of the glandular, fatty and vascular tissue elements, with fatty element with no breast masses seen & no malignant micro calcification detected with no axillary lymphadenopathy and the complimentary sonography revealed absence of any solid or cystic breast masses.

The left breast is showing normal size, shape, nipple, areola, skin and subcutaneous tissue thickness & showed hyper dense lesion at the lateral upper quadrant with marginal speculations and by ultra sound the mass is measuring about (19x14x15mm with 1.5 cm depth) showed posterior acoustic shadowing & noted irregular lobulated outlines and showed mural projections.

Bilateral enlarged axillary LNs

CONCLUSION:

- Left breast suspicious mass (BIRADS IV)*
- Bilateral axillary lymphadenopathy.*

Thanks

DR.Aya Hasan,MSc

BIRADS

BI-RADS (Breast Imaging -Reporting And Data System)

1. Normal
2. Benignfollow up/1 year
3. Probably benign.....follow up/6months
4. Probably malignantfor biopsy
5. Highly suggestive malignant.....for surgery

riadascanlab

فرع الأوراق : ٩٨ كورنيش النيل - بجوار محطة ودانيه

فرع إمبابة: ٧١ شارع الوحدة

٠١١٥٣٠٣٠٣٠٠

Patient Name: Fatma Rezk Ismail Shafie	Date : 12/08/2025
Exam : MAMMOGRAPHY BOTH	Age : 51 Year(s)
Medical ID: 1500/2025	Pat. ID: 100433

**Bilateral Digital soft tissue mammography with high resolution
breast sonography revealed:**

Clinical summary: 51-year-old female with pathologically proven left IDC grade II, hormonal positive subtype. Re-evaluation study.

Findings:

Right breast.

- Scattered fibro-glandular densities. (ACR b)
- UIQ, posterior third, oval circumscribed fat containing lesion.
- No speculations, masses asymmetry , distortion or suspicious microcalcifications.
- Ultrasound demonstrated no solid or cystic lesions.
- 1 o'clock, posterior third non-specific intra-mammary lymph node measuring 7 x 3 mm with 1.7 mm cortical thickness
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left breast.

- Scattered fibro-glandular densities. (ACR b)
- UIQ, posterior third, oval circumscribed fat containing lesion.
- UOQ, middle third , irregular speculated high density mass lesion with distortion yet no suspicious microcalcifications.
- Ultrasound demonstrated 1 o'clock, middle third, 2 cm from nipple, 3.5 mm deep to the skin, irregular anti-parallel spiculated heterogenous hypoechoic solid mass lesion with surrounding echogenic halo, posterior acoustic shadowing and mild internal vascularity on color Doppler study measuring 34 x 18 mm.
- Multiple prominent ducts are seen extending from such lesion to reach the nipple with soft tissue echogenicity within and surrounding abnormal vascularity suggesting anterior ductal extension reaching the nipple.

Patient Name : الأستاذة / فاطمه رزق اسماعيل شافعي
Patient No. : 27409082101184
Path No. : 1382/25
Sex : Female
Age : 50 Y
Referring Doc :

Request No. : 13007747
Request Date : 17/04/2025
Verification Date : 03/05/2025
Financial Category : اوزام دار السلام - مبادره
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

Invasive duct carcinoma, Grade II .

IPt:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Negative (score 0).
- Ki67: Positive in 10% of tumor cells.
- Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.
- Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Prof. Gamal Abdelatty
Dr. Menna Nabil
Dr. Mennan Hossam

Verified by :
Dr.Menan Hossam

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CAP
ACCREDITED

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